

إسم المريض رابعياً : مرفت عبد السلام محمود عامر

الحي / المدينة : اشمون

المحافظة : المنوفيه

رقم البطاقة : 27306111700544 رقم التلفون : 01092219485

العمر(تاريخ الميلاد) : 11-06-1973 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (-----) التوقيع: (-----)

المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني + زولاديكس قسم العلاج :
[14165] الاورام

التعليق على التشخيص : -----

الاسم: د/محمد كيلاني
 الاسم: د/ندى حسن
 الاسم: د/نرمين رمضان
 الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____
 التوقيع: _____
 التوقيع: _____
 التوقيع: _____
 تعتمد صحة البيانات الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
 رئيس اللجنة ومدير
 المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

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قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جـهة العلاج :-

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشئون الصحية

رئیس

المجلس الطبي المتخصص

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Name: مرفت عبد السلام محمود عامر National ID: 27306111700544	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

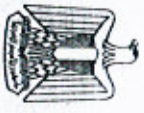
المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Plus Goserline
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٠٢٤/٠٣ ٢٧٣٠١١١٧٠٠٥٤٤
 حاصلة على دبلوم تجارة

إنتى
 رفا محمد عزالقادر محمد
 البطاقة سارية حتى ٢٠٢٧/٠٣/٠٤





مركز التعليم والتدريب
 بطاقة تخفيض الخصخصة

مرقت
 عبد السلام محمد عامر
 قرية طلبة
 مركز الصناعات - المصفاة




٢٧٣٠١١١٧٠٠٥٤٤

KL6688304

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مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية



02/19/2026

MDT FORM

Name:	Mirvat Abd Elsalam Mahmoud	MRN:	4299
Age:	53 Y	ID:	
Menopausal Sta:	post	Phone:	01092219485
NRS SCORE:		Marital Status:	married
BMI:		Family History:	+ve monther
CO Morbidity:	HTN	Reason Of MDT:	

Clinical Data:

BX:	IDC GII
ER:	8/8
PR:	4/8
HER2:	+2(not ar
KI67:	

R

L

Sonomammography:

Mets Workup:

History:

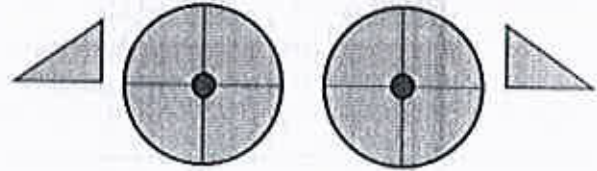
right breast cancer received NACT (4AC +12 W TAXOL) + PHESGO then underwent RT MRM in 2023 then received PORT and tamoxifen 2 years

Recommendations:

for shifting to AI

MDT Outcome & Plan

Shift to AI (femara) +/- zoladex according to menopausal mstatus.
Zometa q6mths.



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

Name: ميرفت عبد السلام محمود		
MR. : 4299		
Detailed Medical Report		
		السيد
تحية طيبة وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته تاريخ وساعة الدخول : تاريخ وساعة الخروج :		
وبعد الكشف والفحوصات والمتابعة وجد انه PATIENT IS 50 YEARS OLD IDC ER:8/8 PR:4/8 HER2: AMPLIFIED RECEIVED NACTH 4AC, 12 WEEKS OF TAXOL, 2 HP RT MRM, COMPLETE PATHOLOGICAL RESPONSE, COMPLETED 1 YEAR OF PHESGO RECEIVED TAMOXIFEN FOR 2 YEARS, E2 LEVEL MENOPAUSAL TO SHIFT TO AI NEXT VISIT breast cancer was on tamoxifen no approval for aramidex yet so for tamoxifen this month BBSM 8/1/2025 >> RT MRM with no local/nodal recurrence + LT deep central asymmetry seen on MLO view for CEM (BIRADS3) PATIENT IN POSTMENOPAUSAL >> FOLLOW UP E2 LEVEL AFTER 3 MONTHS FOR FEMARA ONLY FOR 3 MONTHS		
تحريراً في ,		
	التاريخ :	19-09-2026
	الوقت :	حنان عزت شفيق تاودروس
الطبيب المعالج : يعتمد ,		

- Normal skin thickness and contour.
- Normal nipple areola complex.
- Nonspecific left axillary lymph nodes.

CONCLUSION:

- Right locally advanced cancer breast with insitu components as well as ipsilateral few pathologically enlarged axillary lymph nodes for slide revision if present or another histopathological assessment (BIRADS 6).
- Left breast mammary fibroadenosis (BIRADS 2).

Recommendation - all studies

Referral to clinic

Reported By,

Dr, Salwa Yahya ,MD

18-Apr-2023 11:13 AM

PATIENT NAME: MERVAT ABDUL SALAM MOHMOUD AMER GENDER: Female AGE: 49 y	PATIENT ID #: 10123224454 ORDER #: 2259955,2259956 DATE OF SERVICE: 18-Apr-2023 10:23 AM
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Report: - all studies

DIGITAL MAMMOGRAPHY OF BOTH BREASTS & COMPLEMENTARY
ULTRASOUND

INDICATION:

- Right breast palpable lump (pathologically proven IDC, GII outside Baheya)

FINDINGS:

- Heterogenous fibroglandular parenchyma of both breasts with predominance of nodular densities denoting adenosis which may obscure small masse (ACR c).

RIGHT BREAST:

- Right breast upper quadrants irregular shaped mass lesion is seen associated with regional suspicious microcalcification occupying a total area of about 89x91x102mm along its maximum dimensions (on CC & MLO views).
- Complementary Ultrasound showed right breast upper quadrants diffuse altered parenchyma with area of mass formation seen at 12 o'clock indenting the overlying skin measures about 33x48mm showing internal vascularity.
- Such lesion is seen 26mm away from the nipple.
- Right breast LOQ 7 o'clock another small area of altered parenchyma is also noted.
- Normal nipple areola complex.
- Few (about 3) pathologically enlarged right axillary (Level I) lymph nodes showing irregular focal cortical thickening measures 3.7mm.
- No pathologically enlarged right axillary (Level II, III), internal mammary or supraclavicular lymph nodes.

LEFT BREAST:

- No speculated mass lesions or microcalcific clusters.
- Few scattered punctate loosely attached microcalcification.
- **Complementary Ultrasound showed** no solid or cystic mass lesions.

MRN: 10123224454
Patient Name: MERVAT ABDUL SALAM MOHMOUD AMER
D.O.B: 11/06/1973
IC No.: 27308111700544
Sex: Female
Location: External Clinic

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id 2304300357
Accession No IMM23-2680
Sample Type Paraffin Block
Collection Date 30/04/2023 01:45 PM
Received Date 30/04/2023 01:46 PM
Doctor Nesrin Salah Ahmed Elsabbawy

History, Relevant Clinical Findings & Previous Inv.
Results:

Reason for Order:
..

Immunohistochemical Report

pathology number

3364/23

Clinical Data

Rt. breast cancer

Gross Picture :

Referred block submitted for HER 2 Immunostaining.

System Used :

Ventana BenchMark ULTRA autostainer.

Antibody:

Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :

Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018, IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells, IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in $>10\%$ of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, Intense in $>10\%$ of tumor cells.

Laterality - 1

Right

Her2 Result - 1

Equivocal [score 2+]. Recommended For SISH [available upon request]

Signature

Dr. Ghada M. Abdesalam MD

Reported and electronically endorsed by
Ghada Mohamed Abdulsalam Ahmed

X-RAY REPORT

NAME:	Mrs mervat Monazea kassem
DATE:	Thursday, 16 October, 2025
REF. DR.PROFF.	

CHEST PLAIN X-RAY REVEALED:

- Clear both lung fields with no pulmonary nodules or masses.
- Both costophrenic angles are free.
- Normal cardiac shadow with prominent aortic knuckle.
- Normal bony thoracic cage.

**MUCH OBLIGED;
DR./ Abubakr Hejazi MSc FEBR**

Patient Name: Mervat Monazaa Kasem Monazaa	Date : 03/05/2025
Exam : C.T.P.N.SINUSES	Age : 54 Year(s)
Medical ID:	Pat. ID: 42552

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST & PARA NASAL SINUSES:-

1. PNS:

- Minimal mucosal thickening of both maxillary antra
- Clear scanned rest of paranasal sinuses with intact bony boundaries.
- *No CT evidence of hyper-densities within the mucosal thickening or bone destruction/erosions.*
- No air/fluid levels are seen.
- Occluded left osteomeatal unit
- Patent right osteomeatal units and both sphenoidal recesses.
- The nasal septum is not significantly deviated.
- Clear nasopharynx.

2. CHEST:

. Chest:

****Compared to the last study on 05-12-2024:**

- Regressive size of the left breast retro-areolar ill-defined soft tissue lesion seen inseparable from the skin with diffuse left breast skin thickening mostly post operative sequel.
- Few reticulations noted on lingula likely post-radiation sequel.
- No sizable pulmonary nodules, masses or consolidations.
- Few subcentimetric mediastinal lymph nodes.
- Few prominent axillary lymph nodes noted.
- No evidence of pleural effusion, thickening or masses seen.
- No gross cardiac abnormality. No pericardial effusion detected.
- Normal appearance of the chest wall.
- Upper abdominal cuts are unremarkable.

OPINION:

- *Minimal bilateral maxillary sinusitis.*
- *No CT evidence of pneumonia.*

Dr. Mohamed Mohsen, M.D.

PATIENT NAME: MERVAT ABDUL SALAM MOHMOUD AMER GENDER: Female AGE: 49 y	PATIENT ID #: 10123224454 ORDER #: 2289690 DATE OF SERVICE: 14-May-2023 11:03 AM
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Report:

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY AND
TARGETED US

INDICATION:

- INDICATION:**
- Left breast pathologically proven malignant mass.

TECHNIQUE:

- TECHNIQUE:**
- Contrast Enhanced Spectral Mammography CC and MLO view

REPORT:

- PORT:**
- The breast are heterogeneously dense, which may obscure small lesions (ACR C).
 - Pleomorphic Microcalcifications of regional distribution, seen extending over an area of 10.6x7.3x8.7 cm associated with architecture distortion seen at anterior margin
 - Contrast Enhanced Spectral Mammography showed:
 - Marked symmetric background parenchymal enhancement.
 - Right breast UIQ mass showing high heterogeneous mass enhancement with internal non-enhancing area, associated with posterior non-mass enhancement measuring collectively 10.8x8.1x8.7 cm, reaching to the overlying skin, seen 1.3 cm from the nipple.
 - No left breast significant enhancing lesions.
 - **Previous US revealed :**
 - Few (about 3) pathologically enlarged right axillary (Level I) lymph nodes showing irregular focal cortical thickening measures 3.7mm.

OPINION:

- Right breast UIQ pathologically proven malignant mass and non-mass enhancement as described above (BIRADS 6).
- No left breast suspicious mass or non-mass enhancement (BIRADS 2)



Patient Name: Mervat Abdel Salam Mahmoud Amer	Date : 18/12/2024
Exam : MAMMOGRAPHY BOTH	Age : 87 Year(s)
Medical ID: 1806/2023	Pat. ID: 95497

Left BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had right MRM.

Technique

- Left breast MLO and CC views
- Bilateral ultrasound

Report:

- Heterogenous dense fibroglandular densities of the left breast (ACR C)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions
- Free right infra and supra clavicular regions.

Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Signature Dr. Nehal Hamdy

18/12/2024 1:29 PM

Signature Dr. Nehal Hamdy

18/12/2024 1:30 PM

- aorta, I.V.C and the pelvic organs.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky uterus and cervix with distension of the endometrial cavity and cervical canal with thickened heterogenous contents with also distension of the upper vagina.
- Left adnexal thin walled hypodense cyst measuring 41x27 mm.
- Bilateral para metrial dilated vascular channels are noted along with lower anterior abdomen subcutaneous varicosities.
- No pathologically enlarged abdominal or pelvic lymph nodes.
- No abdominal or pelvic masses or collections.

The revised bone window revealed:

1. No suspicious osseous lytic or sclerotic lesions.
2. L5/S1 grade I lytic spondylolisthesis.

OPINION:

- ***Right breast upper central malignant looking lesion with ipsilateral prominent axillary lymph nodes.***
- ***Hepatomegaly with two hepatic focal lesions as described likely hemangiomata as well as segment VI small flash filling hemangioma for further MRI evaluation.***
- ***Bulky uterus and cervix with distended endometrial cavity and cervical canal with heterogenous contents for further CE-MRI assessment.***
- ***No CT evidence of pulmonary or osseous metastasis.***

Reported By,

Dr. Ali Saber, MSc
Dr, Salwa Yahya ,MD

13-May-2023 04:48 PM

Name: Mervat Abd El Salam Mahmoud

Date: 30/10/2023

LV measurements	Patient		Left atrium, Rt. side and aorta	Patient
LVEDD	50		Aortic root	35
LVESD	32		Cusp opening	
EF%	65			
FS	36		Left atrium	38
IVST	8			
PWT	8		RV	

COMMENT

Left ventricle: Normal LV internal dimensions.

Normal LV systolic function (EF65%).

No resting wall motion abnormalities.

Diastolic dysfunction grade I.

Left atrium: Normal.

Mitral valve: Slightly thickened and prolapsed. There is mild mitral regurge.

Aortic valve: Normal leaflets thickness, motion and excursion. There is no significant stenosis or regurge.

Pulmonic valve: Normal leaflets thickness, motion and excursion. There is trivial pulmonic regurge.

Tricuspid valve: There is mild tricuspid regurge with the predicted pulmonary artery systolic pressure is 28mmHg.

Right ventricle: Normal dimensions and contractility.

Pericardium: Normal thickness. No pericardial effusion.

Great vessels: Aorta is of normal size. Pulmonary artery and both branches are of normal size.

The interatrial septum: Intact.

The interventricular septum: Intact.

No intracardiac masses or thrombi.

Dr. Ahmad Gabr., MD Cardiology
National heart institute

PATIENT NAME: MERVAT ABDUL SALAM MOHMOUD AMER GENDER: Female AGE: 49 y	PATIENT ID #: 10123224454 ORDER #: 2289672,2289673 DATE OF SERVICE: 11-May-2023 03:52 PM
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Report: - all studies

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN
AND PELVIS**

1-CHEST:

- Right breast upper central irregular shaped heterogeneously enhancing mass lesion is seen measuring about 37 x 31mm seen reaching the skin along with associated upper quadrant enhancing nodules and heterogenous parenchyma.
- Few mildly enlarged right axillary (Level I & II) lymph nodes, the largest measuring 17x8mm in axial planes.
- Bilateral basal atelectatic bands.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitory lesions.
- No CT evidence of pathologically enlarged hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers.
- No pericardial effusion detected.

2- ABDOMEN AND PELVIS:

- The liver is enlarged with homogenous CT texture. It shows two hypo enhancing focal lesions seen at segment VIII and II showing peripheral nodular enhancement and become isodense on delayed phase. Largest at segment VIII measuring 22x14mm.
- Another segment VI small arterial intensely enhancing focal lesion is also noted measures about 6x8mm likely denoting flash filling hemangioma.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- Normal appearance of the pancreas, spleen, kidneys, adrenals,